

Illnesses and medication

Knowing how to manage illness and what medications are safe to take is important to protect your own and your baby's health.

If you have an existing medical condition, seek advice from your doctor before you conceive as some medications may need to be changed before becoming pregnant.

Preexisting conditions

If you have a condition such as high blood pressure, asthma, epilepsy, or diabetes, your pregnancy will be classified as high risk and you'll need to be monitored carefully.

Women with high blood pressure, kidney disease, diabetes, systemic lupus erythematosus, previous preeclampsia, a BMI greater than 35, or a maternal age over 40 may be advised to start taking low-dose aspirin between 12 and 16 weeks of pregnancy. If you become pregnant while taking medication for a condition, don't stop taking the medication; consult your doctor as soon as possible. Your medication may be safe, or you may need to change to another type of medication. The most important thing is to control your condition during pregnancy to minimize the risks to you and your baby, which will usually mean continuing with medication.

Diabetes If you have diabetes and are planning to conceive, discuss with doctor the best way to control your blood sugar levels and how you will manage diabetes in pregnancy. Women with diabetes are advised to take a prenatal vitamin with folic acid before trying to conceive and for the first three months of pregnancy. Diabetic women who are overweight may be advised to lose weight before getting pregnant, and they'll likely be told to monitor their blood sugar more frequently and take their medications on time during their pregnancy. Babies born to diabetic women also have a greater risk of higher birth weight, respiratory problems at birth, jaundice, and low blood sugar at birth.

As soon as you're pregnant, you should be referred to an obstetrician who specializes in pregnancy and diabetes, where you'll receive extra care. You will have more frequent prenatal visits, additional scans, and extra blood tests to monitor your blood sugars. You may need diabetes medication and/or insulin injections each day; the dose usually changes throughout pregnancy and needs to be monitored. The better your blood sugar control, the less likely you or your baby is to experience problems during pregnancy.

Since diabetic women have an increased risk of late pregnancy problems such as [preeclampsia](#) and [premature labor](#), you may be advised to have an [induction of labor](#) before your due date. The exact timing will depend on factors such as your blood-sugar control during pregnancy and the size of the baby. Once in labor, your blood sugar levels will be closely monitored, and you will probably be given an insulin and glucose IV. After the birth, your baby's blood sugar levels will be closely monitored, too, for 24 hours. If you're planning to breastfeed, your insulin dose may need to be changed after the birth.